

Name : **Saroj Agarwal** Age/Gender : 63 y / F Collected : 20/02/2026 05:17
Patient ID : **0010663186** Visit Type : IP Received : 20/02/2026 10:06
Referred By : Dr. Darshit Shah DOB : 27/08/1962 Reported : 23/02/2026 12:06
Accession : 2600040492 Location : 1T06N`1TR0601`1TB0605`1ONCOMED

DEPARTMENT OF LABORATORY MEDICINE - MICROBIOLOGY

Test Name

Result



Urine Culture Colony Count

Report Status Final report

Clinical Details Breathlessness

Sample Urine

SITE Foleys

Microscopy
Wet mount:
Pus Cells: >100/hpf
Red Blood Cells: >100/hpf
Epithelial Cells: 0-5/hpf
Bacteria : Present

Culture
1. Enterococcus faecium: Colony Count: >10⁵
CFU/mL: Significant bacteriuria
2. Candida tropicalis: Colony Count: >10⁵
CFU/mL: Significant growth

Organism
1. Enterococcus faecium
2. Candida tropicalis

DRUG SUSCEPTIBILITY

Candida tropicalis

Antimicrobial Agent	Interpretation	MIC Value (µg/ml)
Fluconazole	Susceptible	1

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DRUG SUSCEPTIBILITY

Candida tropicalis

Antimicrobial Agent	Interpretation	MIC Value (µg/ml)
Voriconazole	Susceptible	<=0.12
Caspofungin	Susceptible	<=0.12
Micafungin	Susceptible	<=0.06
Amphotericin B	Susceptible	0.5

DRUG SUSCEPTIBILITY

Enterococcus faecium

Antimicrobial Agent	Interpretation	MIC Value (µg/ml)
Ampicillin	Resistant	
Benzylpenicillin	Resistant	>=64.0
Ciprofloxacin	Resistant	>=8.0
Daptomycin	Susceptible- Dose dependent	1.5
Levofloxacin	Resistant	>=8.0
Linezolid	Susceptible	2.0
Nitrofurantoin	Resistant	256.0
Teicoplanin	Susceptible	<=0.5

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DRUG SUSCEPTIBILITY

Enterococcus faecium

Antimicrobial Agent	Interpretation	MIC Value (µg/ml)
Tetracycline	Resistant	≥ 16.0
Vancomycin	Susceptible	≤ 0.5

Comment

Enterococcus faecium is inherently resistant to Cephalosporins, Aminoglycosides, Clindamycin, Trimethoprim, Trimethoprim-sulfamethoxazole and Fusidic Acid. An interpretation of susceptible for ampicillin in beta- lactamase negative enterococci is also applicable to amoxicillin-clavulanate, ampicillin- sulbactam, piperacillin and piperacillin- tazobactam among non-β-lactamase producing enterococci. Combination therapy of ampicillin, penicillin, or vancomycin (for susceptible strains) plus an aminoglycoside, is usually indicated for serious enterococcal infections, such as endocarditis, unless resistance to both high- level gentamicin and high- level streptomycin is documented; such combinations are predicted to result in synergistic killing of enterococcus.



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Remarks

Identification and antimicrobial susceptibility performed on automated system/s
Disc diffusion method/ software deduced interpretation is followed where MIC value is not reported
Antibiotic susceptibility reported as per CLSI guidelines.



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Method: Specimen processed on culture media. Identification by automated MALDITOF / Vitek2 & or conventional methods, sensitivity on Vitek 2. Certain antibiotics tested by Kirby Bauer's method and MIC value is not reported for antibiotics which are deduced by software as per CLSI Guidelines.

Remark:

Note:

1. Result of culture and antimicrobial susceptibility test need to be correlated clinically.
2. Previous history of antibiotic usage may influence the growth of microorganisms in vitro.
3. Sterile pyuria is a condition in which the pus cells/ white cells are present in the urine without bacterial infection.
4. Asymptomatic bacteriuria, is a condition in which bacterial count may be significant but patient does not experience any symptoms.

Colony Count	Interpretation
Colony Counts of 10000 to ≥ 100000 CFU/ml of single/two Potential pathogen/s.	Significant uropathogen. Suggestive of Urinary tract infection (UTI) needs treatment based on antimicrobial susceptibility test results.
Colony counts between 1000 to 10000 CFU/ml of single Potential pathogen.	Can be considered Significant growth, correlation with Microscopy and Clinical history required.
Colony counts between 100 to 1000 CFU/ml.	Insignificant growth. Probable commensal contamination during voiding.
Either two or three organism types with predominant growth of one organism type and <10000 CFU/mL of the other organism type(s)	Suggestive of Urinary tract infection (UTI) needs treatment based on antimicrobial susceptibility test results. Correlation with Microscopy and clinical history required
Any number / Any count.	Significant in case of Suprapubic aspirates/surgically obtained (e.g. cystoscopy) urine specimens.
≥ 3 organism types with no predominant (10000 ≥ 100000 CFU/ml) pathogen.	Fresh specimen required as possibility of contamination during voiding.

Reference:

1. Bailey & Scott's Diagnostic Microbiology 15th Edition.
2. CLSI Guidelines.

∞ End of Report ∞

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